DISCHARGE INSTRUCTIONS - CARDIOLOGY

Patient: WASHINGTON, DELORES A. MRN: 30918827 DOB: 06/30/1950
Admitted: 10/18/2026 Discharged: 10/23/2026 Attending: S. Brandt, MD

DIAGNOSIS: Acute on chronic systolic congestive heart failure (HFrEF), EF 30%. Precipitant: dietary indiscretion and medication non-adherence.

HOSPITAL COURSE: Admitted with bilateral lower extremity edema, orthopnea, and PND. BNP 1,840 pg/mL. Diuresed with IV furosemide, net negative approx 6.2 L. Weight on admission 198 lb; discharge (dry) weight 191 lb.

MEDICATIONS AT DISCHARGE:
- Furosemide 40 mg PO daily (INCREASED from 20 mg)
- Carvedilol 6.25 mg PO BID w/ food (CONTINUE)
- Sacubitril-valsartan 24/26 mg PO BID (NEW - replaces lisinopril; STOP lisinopril)
- Spironolactone 25 mg PO daily (NEW)
- Potassium: do NOT take supplements unless instructed. Labs will be rechecked.

SELF-MONITORING:
1. Weigh yourself every morning after voiding, before breakfast. Record it.
2. CALL your cardiologist if you gain >3 lb in 1 day or >5 lb in 1 week.
3. Limit sodium to <2,000 mg/day. Limit fluids to 64 oz (2 L)/day.
4. Report increased swelling, new cough at night, needing extra pillows to sleep, or dizziness.

AVOID: NSAIDs (ibuprofen, naproxen), salt substitutes containing potassium chloride.

FOLLOW-UP: Cardiology clinic (Dr. Brandt) in 7 days. BMP (kidney function and potassium) in 1 week. Home health nurse visit within 48 hours.
CALL 911 for: severe shortness of breath at rest, chest pain, fainting.